جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : سهير مختار عبدالهادى بحبح
المحافظة : القليوبيه **الحي / المدينة :** شبرا الخيمه اول **العنوان :** +++++++
رقم البطاقة : 28309011601541 **رقم التليفون :** 01205590445 **تاريخ تسجيل الإستمارة :** 20-10-2026
العمر(تاريخ الميلاد) : 01-09-1983 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي العلاج المساعد لوقاية العظام للمرضي الذين يتلقوا علاج هرموني حمض الزيلودرونيك [14157] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: سهير مختار عبدالهادى بحيح
National ID: 28309011601541

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Zoledronic acid for bone protection during hormonal therapy

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



٢٨٣٠٩٠١١٦٠١٥٤١ ٢٠٢٣/٠٨

ربه منزل

أنثى مسلمة متزوجة

ابراهيم محمود حسن احمد

١١ البطاقة سارية حتى ٢٠٣٠/٠٨/٢٨



جمهورية مصر العربية

بطاقة تحقيق الشخصية

سهير

مختار عبدالهادى يوسف بحيح

٤٨ش عبدالمقصودابوشنب-الفرنوانى

شبرا الخيمة أول - القليوبية

٢٨٣٠٩٠١١٦٠١٥٤١

KD4367193

National ID - Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

Patient Name	: الأستاذه / سهير مختار عبدالهادى يوسف بحيح	Request No.	: 136117
Patient No.	: 28309011601541	Request Date	: 13/11/2021
Path No.	: 2731/21	Verification Date	: 22/11/2021
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 38 Y	Department	: Out Patient
Refereing Doc :			

Pathology Report

Clinical Data: Breast lesion

Nature of the speciment: Biopsy
Gross Examination: Seven small grayish white tissue cores, up to 0.4 cm long. Totally submitted.
- Procedure: Tru cut
- Localization: Right

Microscopic examination:
- Histological type: Skin and blood clots
- Grade : N/A
- Angioinvasion: No
- Ductal carcinoma in situ: No

Diagnosis:
- Right breast lesion, Core biopsy;
- Skin and blood clots, if malignancy is suspected another biopsy is recommended.

Prof.Dr. Gamal Abd El Aty
Dr.Mona Mohamed

Room No. (4) - First Floor- Building No. 397- Nile Corniche - Nasr El Qadeem - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Name: سهير مختار عبد الهادي يوسف	
MR. : 6237	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		**بشأن تقرير طبي للمريض**	
بقسم:		**رقم طبي:**	

ننشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		**تاريخ وساعة الخروج :**	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient was initially diagnosed with primary breast cancer with hepatic metastases, triple-positive disease, and was started on a regimen of gemcitabine (GEMZAR) and carboplatin (CARBO) at cycle 4 (C4).

Subsequently, the disease was noted to be locally recurrent breast cancer with pulmonary metastases, still triple-positive, and the patient was placed on a HERCEPTIN-containing regimen at cycle 6 (C6).

The patient received six cycles of HERCEPTIN, the last dose administered on 31/12/2025. Imaging in December 2025 demonstrated progression of multiple osseous nodules, regression of pulmonary nodules, and stable disease elsewhere; a bone scan was negative. HERCEPTIN therapy was suspended pending a whole-spine MRI, and the patient was continued on aromatase inhibitor (Aromasin) and Zoladex while awaiting MRI results.

After counseling, the patient agreed to receive HERCEPTIN intravenous, capecitabine (XELODA), and zoledronic acid (ZOMETA) as palliative therapy for bone lesions.

The team considered adding tyrosine-kinase inhibitor (TYKERB) and navilbine in the event of disease progression on HERCEPTIN and XELODA.

The patient's history includes right-sided breast cancer (T2 N0/15, triple-positive) treated with three cycles of AC, three cycles of taxane, and HERCEPTIN plus tamoxifen until November 2021. A local recurrence was treated with taxane, carboplatin, and HERCEPTIN in October 2023. Subsequent imaging in May 2026 showed multiple bilateral pulmonary nodules (largest 2.5 × 3.5 cm) and a hemangioma, as well as cervical, dorsal, and lumbar osseous deposits on MRI. The patient was prescribed HERCEPTIN 440 mg on day 1 (C7) and capecitabine 4 g in the morning and 3 g in the evening for 14 days, but the patient declined further intravenous chemotherapy and has been off treatment since December 2025.

For cycle 8 (C8) the patient received HERCEPTIN on day 1 and capecitabine 4 g morning/3 g evening for 14 days; the patient reported gastric pain, pending echocardiogram, and was to receive ondansetron and gastric support after laboratory work.

For cycle 9 (C9) the patient received HERCEPTIN intravenous, liver support, and capecitabine 3 g three times daily for 14 days; laboratory results were accepted.

The patient planned an Umrah trip and wished to pause capecitabine during travel to avoid treatment complications, with a consultation by Dr. Shaimaa. The decision was made to hold capecitabine while continuing HERCEPTIN for cycle 10 (C10). The patient has now completed ten cycles of HERCEPTIN.

تحريرا في ,

التاريخ :	18-09-2026	**الوقت :**	

الطبيب المعالج : dina makram

يعتمد ,

مركز أورام دار السلام
إداره المعمل
قسم الباثولوجى

مركز أورام دار السلام

Pathology Report

Name: سهير مختار عبد الهادى **Age: 35 years.**

Clinical data: right breast cancer

Specimen Type: Lumpectomy & axillary clearance.

Date: 30 /1/2019

Lab. Number: 57 **Hospital Number: /2019**

Gross Description:

Fibrofatty piece; 15.5x9.5x3 cm, with attached skin ellipse; 6.5x1.5 cm. Careful dissection revealed an ill-defined mass ; 2.7x2 cm. and located 2, 1, 2.5, 11 , 2, 4.5cm away from skin, deep, medial, lateral, superior, inferior respectively. Separate axillary tissue; 15x9 cm, entangling 12 nodules largest 2x1.5 cm.

Microscopic Examination:

Histopathological examination of sections prepared from the tumour mass reveals an infiltrating malignant epithelial growth made up of groups and sheets of pleomorphic hyperchromatic malignant ductal epithelial cells. Stromal desmoplasia is moderate. Cellular and nuclear atypia are moderate (grade II). No intraductal component. All of the 15 lymph nodes detected show only reactive hyperplasia (Nodal score is 0/15). The nipple, areola and surgical margins are free.

Diagnosis:

Lumpectomy & axillary clearance, Invasive duct carcinoma GII

- **Free margins.**
- **Free lymph nodes 0/15.**
- **Stage: T2 NO Mx.**

Prof. Dr. Amr El-Sebaie

Dr. Marwa Mahmoud

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

GUSTAVE ROUSSY
INTERNATIONAL

Patient Name	:الأستاذه / سهير مختار عبدالهادى يوسف بحيح	Request No.	: 13008677
Patient No.	:28309011601541	Request Date	: 29/07/2025
Path No.	:2403/25	Verification Date	: 03/08/2025
Sex	:Female	Financial Category	: جوستاف روسى - نفقة الدولة
Age	:41 Y	Department	: Out Patient
Referreing Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical data
Right dermis lesion , post MRM .

Nature of specimen
Wedge biopsy.
Gross examination
skin ellipse measured 1`.7x1.5 with subcutis , 1.2 cm thick .It showed a congested nodule measured 0.4x0.4 cm , least margin 0.1 cm, all around . Also received sparately in the same container askin covred tissue piece measured 1x1 cm,. totally submitted.

Microscopic examination
Examination revealed busy dermis, the dermis is infiltrated with malignant cells with high degree of pleomorphism and hyperchromatic nuclei , free epidermis
-Deep margin is free .
-Positive other surgical margins .
-Positive lympho-vascular emboli .

Diagnosis
Right dermal lesion, wedge biopsy :
Invasive duct carcinoma grade 3,.
-All Side margins are infiltrated , but deep margin is free .

Signature
Prof. Dr. Gamal Abdelatty
Dr.Menan Hosam

Verified by :

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

atient Name	: الأستاذه / سهير مختار عبدالهادى يوسف بحيح	Request No.	: 13009010
atient No.	: 28309011601541	Request Date	: 29/10/2023
ath No.	: 3748/23	Verification Date	: 12/11/2023
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 40 Y	Department	: Out Patient
Referreing Doc :		Registered by	: Ali mostafa

Pathology Report

Clinical Data:
ight breast cancer

Nature of the specimen:
Right completion mastectomy

Gross Examination:
- Procedure: Right Completion mastectomy specimen measured 21x16x3 cm with covering skin ellipse measured 19x13 cm .
- Localization: Central and UOQ
- Tumor extent : A greyish white granular tumour bed site
- Size: 5x3x1 cm
- Skin involvement: Nipple & areola were unremarkable grossly
- Distance from margins: 1 cm away from deep margin

Microscopic examination:
- Histological type:Microscopic foci of Invasive duct carcinoma
- Grade (differentiation+ nuclear pleomorphism + mitosis):

Grade II (3+2+1)
- Angioinvasion:Positive
- Ductal carcinoma in situ: No
- Skin involvement: Free nipple and areola
- Resection Margins: Free margins
- Histologic response to therapy: Pathologic poor response

Diagnosis:
Right completion Mastectomy :
Microscopic foci of Invasive duct carcinoma grade II with frequent lymphatic tumor emboli

Verified by :
Dr.Heba Hasan

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

ent Name	: الأستاذه / سهير مختار عبدالهادى يوسف بحيح	Request No.	: 13009010
ient No.	: 28309011601541	Request Date	: 29/10/2023
n No.	: 3748/23	Verification Date	: 12/11/2023
x	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
e	: 40 Y	Department	: Out Patient
ferreing Doc :		Registered by	: Ali mostafa

Pathology Report

nd totally necrotic DCIS component
Free margins
Free nipple and areola
poor pathological response
Stage : ypT1mi

Signature

Prof. Dr.Amal Hareedy
Dr.Heba Hassan

Verified by :
Dr.Heba Hasan

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Sohair Mokhtar Abdel Hady Youssef Bahbah	Date : 06/05/2025
Exam : MAMMOGRAPHY BOTH	Age : 41 Year(s)
Medical ID: 73/2019	Pat. ID: 21939

DIGITAL SOFT TISSUE MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND

Indication:

Patient is 41years old and is seen for follow-up after right MRM Complaining of right para-sternal dermal lesion (red and slightly raised and indurated)

Report:

- Right MRM:
  - Operative bed shows no mass or collection
  - Dermal lesion of concern shows no underlying mass
  - No regional lymphadenopathy.
- Left Breast:
  - Scattered fibroglandular parenchymal densities (ACR b)
  - No suspicious micro calcifications or architectural distortion detected.
  - Complementary ultrasound no cystic or solid mass lesions.
  - Normal skin thickness, and nipple-areola complex with average caliber of the retro-areolar central ducts.
  - Nonspecific axillary LN.

Opinion:

- Post right MRM status showing no evidence of recurrence (BIRADS 2) Dermal lesion for clinical and dermatology assessment and follow-up after treatment
- Normal sonomammography of left breast (BIRADS 1)

Best regards,
Dr. Amira H Radwan MD,
Breast Imaging consultant,
NCI, Cairo University
06/05/2025 2:19 PM

Mammogram
[[DOCTYPE:MAMMOGRAM]]

GUSTAVE ROUSSY
INTERNATIONAL

Name : Sohair Mokhtar Abdel Hady Youssef Bahbah	Date : 17-12-2025 12:56
Branch : Portable	Medical No. : GRI-21939
Ref.Physician Prof :	Ins.Company :

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

Compared to previous study dated 18-8-2025 the current study shows:

1. ***Chest:***

- Right mastectomy with clear operative bed , no masses or collection.
- Regressive course regarding the size and number of the previously noted pulmonary nodule, the lingular nodule seen measuring 5 mm compared to 11 mm in the previous study.
- Still noted few paraseptal emphysematous bullae , the largest at middle lobe measures 2 x 1 cm.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Average cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

2. ***Abdomen and pelvis:***

- The liver is average in size with homogenous CT texture. No appreciable time interval changes regarding the size of the previously noted segment IVb and segment II hypoense focal lesions, the largest at segment IVb showed faint nodular enhancement in arterial phase.
- Patent homogenously enhancing portal vein and is main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.

CT Scan
[[DOCTYPE:CT_SCAN]]

GUSTAVE ROUSSY
INTERNATIONAL CANCER

Name : Sohair Mokhtar Abdel Hady Youssef Bahbah	Date : 17-12-2025 12:56
Branch : Portable	Medical No. : GRI-21939
Ref.Physician Prof :	Ins.Company :

- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both supra-renals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.
- Still noted L2 tiny radiolucent lesion.
- Newly developed multiple vertebral bodies sclerotic lesions

OPINION:Follow up study shows :
Progressive course regarding the multiple osseous nodules.
Regressive course regarding the pulmonary nodules.
Stationary course regarding the rest of the study :

Dr. Asser Ahmed

Dr. Asser Ahmed

CT Scan
[[DOCTYPE:CT_SCAN]]

Name : Soheir Mokhtar Abdelhadi Youssef
Branch : Dokki
Ref.Physician Prof : Goustav Roussey Egypt

Date : ٠٨-٠١-٢٠٢٦ ١٠:٤٢
Medical No. : ٢٩٠٩٨٥
Ins.Company : تعاقد مستشفيات

NILE DIAGNOSTIC CENTER

Tc^{99}m MDP SKELETAL SCINTIGRAPHY

History

- A ٤٢-year-old lady, with history of right breast cancer and referred for metastatic workup.

Technique

- Anterior and posterior whole-body scans were acquired ٢ hours post injection of the tracer.

Findings

- No abnormal active or cold focal bony lesions that could account for metastatic bone disease.
- Degenerative changes are noted at the spine and big joints.

Impression:

- **The skeleton is free from metastases.**

Best regards,
Dr. Alaa Nagi, MD
Nuclear Medicine

Dr.Mohamed Hassan

Dr. Alaa Nagy

Bone Scan
[[DOCTYPE:BONE_SCAN]]



Dms
توقيع
توقيعات



National ID · Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

[[DOCTYPE:MDT_FORM]]
MDT Form

جوستاف روسي

مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

البنك المصري لتنمية الصادرات
صحة مصر

10/18/2026

MDT FORM

Name:	Souhair Mukhtar Mohamed Yousef	**MRN:**	6237
Age:	43 Y	**ID:**	28309011601541
Menopausal Sta	prem	**Phone:***	01205590445
NRS SCORE:		**Marital Status:**	
BMI:		**Family History:**	
Gender			
CO Morbidity:		**Reason Of MDT:**	quirry dp
Surgical History:*	mrm	**ECOG Performance Status**	○ 0 ◉ 1 ○ 2 ○ 3 ○ 4

Clinical Data:

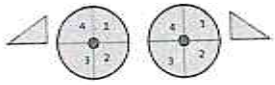
BX:

ER: **PR:** **HER2:** **KI67:**

History:

prem locally recurrent rt breast cancer met to lung bone triple positive history rt bcs t2 n0/15 triple positive rec 3 ac 3 txt herceptin tamoxifen till 11/2021 local recurrence rec taxol carbopl herceptin on 10/2023 rt mrm ypt1 continued phesgo zoladex ai till 7/2025 rt local recurrence mets to bone lung rec taxol herceptin iv last 12/2025 did not rec tt from 12/2025 till6/2026 ,ef 62%12/2025 cts 5/2026 multiple bilat pulmonary nodules largest m 2.5x3.5 med ln 18x14 hfl hemangioma , mri multiple cervical dorsal lumbar osseous deposits . pt resumed herceptin and st xeloda from 6/2026 pt rec 4 cycles xeloda bs no comp bs 1/2026 free althoug mri 5/2026 cervical dorsal lumbar vertebrae BS 6/10/2026:widespread multifocal active osseous lesions throughout axial and appendicular osseous lesions skull,cervical ,dorsal lumbar pelvic bone .

R L



TNM

Right Side				Left Side			
Q1	Q2	Q3	Q4	Q1	Q2	Q3	Q4

Clinical Staging:

Pathological Staging:

Sonomammography:

Biomarker/Molecular

Mets Workup:

cts 29/9/2026 comp to 5/2026 sd of 2 hfl mostly hemangioma no pulmonary nodules ,newly developed multiple vertebral sclerotic lesions suspicious pd newly dev osseous lesions for MDT

Recommendations:

MDT Outcome & Plan

for tykerb navelbin
zometa every 3 ms after dental assessment

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
د.حنان عزت	د احمد نجي		